

Visit checklists

Each checklist lists the visit's procedures in the order to run them, with the worksheet that captures each one. The worksheets (codes W1 to W19) are not included in this sample.

Every visit

These three lists apply to every visit and are not repeated below.

The day before

- ☐ Visit date is inside the window, counted from Day 1
- ☐ Last visit's note reviewed: open adverse events, pending lab results, dose changes, open queries
- ☐ Worksheets printed with only the header filled in. Never pre-fill data
- ☐ Lab kit for this visit on hand and in date. ECG machine, scale and thermometer working
- ☐ Study drug in stock and IWRS login working
- ☐ Investigator booked for the exam, C-SSRS review and sign-off
- ☐ Reminder call made: bring the seizure diary and every card or bottle, including empties

Order during the visit, and why

1. Diary, adverse events and concomitant medications first. The investigator needs them before the exam.
2. Questionnaires next, before any procedure. Pain, fatigue or bad news changes how people answer.
3. Vital signs after 5 minutes supine, then the ECG. Both need a rested subject.
4. Exams, then blood and urine. A needle stick raises heart rate and blood pressure.
5. Drug accountability, IWRS, dispensing and dosing instructions last, so new cards reflect today's decisions.

Before the subject leaves

- ☐ New seizure diary handed over and recording rules repeated
- ☐ Subject says the dosing instructions back to you
- ☐ Next visit booked inside its window
- ☐ Subject reminded to call the site at once for any rash, new medication or suspected pregnancy

After the visit

- ☐ Every worksheet field is completed, or lined through with a reason, initialed and dated
- ☐ Investigator has signed exams, ECG and each adverse event's severity and causality
- ☐ eCRF entered. Medication numbers match between cards, IWRS and eCRF
- ☐ Samples shipped. Lab reports signed by the investigator when they return, with each abnormal value assessed
- ☐ Any serious adverse event reported within 24 hours of awareness
- ☐ Any protocol deviation logged and reported

Screening and baseline: Visits 1 to 3

No study drug is given in these 8 weeks; their purpose is to prove eligibility and to count seizures.

Visit 1, Day -56 (±2 days): screening

- ☐ Informed consent signed before any study procedure, copy given to the subject. Assent as well if a guardian consents (W1)
- ☐ Screening number assigned: 8 digits, 2 for country, 3 for site, 3 for sequence
- ☐ Demographics, medical history, seizure history and AED history (W2)
- ☐ Concomitant medications with doses and start dates, checked against the prohibited list (W14)
- ☐ Adverse events recorded from the moment of consent (W15)
- ☐ C-SSRS, Baseline/Screening version, reviewed by the investigator before the subject leaves (W11)
- ☐ Vital signs: blood pressure and heart rate supine after 5 minutes, then standing at 3 minutes. Respiratory rate and oral temperature supine (W3)
- ☐ Height and weight. Height is measured only at this visit (W3)
- ☐ Triplicate 12-lead ECG, at least 2 minutes between recordings (W7)
- ☐ Full physical exam (W4)
- ☐ Full neurologic exam (W5)
- ☐ Blood for hematology and chemistry. Serum pregnancy test for women of childbearing potential (W8)
- ☐ Urine for urinalysis (W8)
- ☐ EEG report filed in source. If none can be obtained and the subject is otherwise eligible, an EEG may be performed
- ☐ CT or MRI report from the past 10 years filed. If none, imaging ordered so it is done before randomization, with a copy of the report for the sponsor
- ☐ Inclusion and exclusion criteria reviewed, first pass (Eligibility checklist)
- ☐ Diagnostic Review Form completed, signed by the investigator and sent to the independent reviewers right after the visit
- ☐ Seizure diary dispensed. Subject and caregiver trained to record every seizure by type and number (W10)
- ☐ Subject told: keep AED doses unchanged, no diazepam, at most 1 rescue benzodiazepine use during baseline
- ☐ Visit 2 scheduled in 4 weeks

After the visit, when labs return: calculate creatinine clearance by Cockcroft-Gault, have the investigator sign the report, and answer the lab-based exclusion rows. Track Diagnostic Review Form queries until the approved form is back; no approval, no randomization.

Visit 2, Day -28 (±2 days): baseline midpoint

- ☐ Seizure diary collected and reviewed with the subject for completeness and accuracy (W10)
- ☐ Qualifying seizures in weeks 1 to 4 counted. Fewer than 3 means the subject cannot meet inclusion 6, so tell the investigator today
- ☐ Adverse events (W15)
- ☐ Concomitant medications, with AED doses confirmed unchanged (W14)
- ☐ Vital signs (W3)
- ☐ Outstanding screening items chased: Diagnostic Review Form, imaging, EEG report, repeat labs
- ☐ New seizure diary dispensed
- ☐ Visit 3 scheduled in 4 weeks. Subject told it is a long visit and to note the time of that morning's AED doses

Visit 3, Day 1 (±2 days): randomization

Finish eligibility before anyone opens IWRS. Labs drawn today are the safety baseline; they are not back in time to decide eligibility, which rests on Visit 1 labs.

- ☐ Seizure diary collected. Baseline seizure count completed and recounted by a second person (Eligibility checklist)
- ☐ Adverse events and concomitant medications. AED doses unchanged since Visit 1 (W15, W14)
- ☐ QOLIE-31-P completed by the subject alone, never by a caregiver. Not within 4 hours of a simple motor or complex partial seizure, or 24 hours of a generalized seizure, unless fully recovered (W11)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ Single 12-lead ECG (W7)
- ☐ Brief neurologic exam (W6)
- ☐ Urine pregnancy test. A positive result is confirmed by serum and the subject is not randomized (W8)
- ☐ Blood for hematology and chemistry, and urine for urinalysis (W8)
- ☐ Blood for concomitant AED levels, drawn before any study drug, with the time of the last AED doses recorded (W9)
- ☐ Inclusion and exclusion criteria re-checked, approved Diagnostic Review Form on file, investigator signature with time (Eligibility checklist)
- ☐ Randomized in IWRS. Randomization number, date and time recorded (W12)
- ☐ 2 blister cards dispensed as IWRS directs. Each label completed: subject number, study week, date dispensed, site number, visit number, phone contact, investigator name. Dose dates written beside the day numbers (W13)
- ☐ Dosing taught: first dose tomorrow morning, Day 2. Once daily in the morning, with or without food, tablets swallowed, kept in the original card
- ☐ If the investigator wants to observe the first dose in clinic, it is given after every other Visit 3 procedure. Seizures after that dose are counted
- ☐ New seizure diary dispensed. Seizures on Day 1 are not counted
- ☐ Visit 4 scheduled in 2 weeks and the Day 8 phone call booked

Titration: Visits 4 to 6 and the three phone calls

The blinded dose can rise each week for 6 weeks, so tolerability is the question at every contact. The dose rules are on the Synopsis tab.

Phone calls, Days 8, 22 and 36

- ☐ Tolerability asked about with open questions, then rash asked about by name
- ☐ Adverse events recorded with onset date. Investigator told the same day (W15)
- ☐ Compliance: doses taken on the dates written on the card, any missed doses
- ☐ New medications or AED changes since the last contact (W14)
- ☐ Diary being filled in daily
- ☐ Any rash reported: prompt unscheduled visit arranged, with hematology and chemistry
- ☐ Tolerability problem that may need a dose change: unscheduled visit arranged. A dose is never changed by phone, and never reduced in week 1
- ☐ Call documented: date, time, who you spoke to, failed attempts (W16)

Visit 4, Day 15 (±2 days)

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events (W15)
- ☐ Concomitant medications. Rescue benzodiazepine uses tallied: at most 2 during the treatment phase (W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs (W3)
- ☐ Brief neurologic exam (W6)
- ☐ Drug accountability: tablets left on each returned card counted and compared with expected. Differences explained by the subject and written down (W13)
- ☐ Investigator's tolerability decision recorded. Any dose change made through IWRS at this visit (W17)
- ☐ Cards dispensed as IWRS directs, expect 2. Labels and dose dates completed (W13)
- ☐ New seizure diary dispensed
- ☐ Visit 5 scheduled in 2 weeks and the Day 22 phone call booked

Visit 5, Day 29 (±2 days)

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ Brief neurologic exam (W6)
- ☐ Drug accountability (W13)
- ☐ Investigator's tolerability decision. Any dose change through IWRS (W17)
- ☐ Cards dispensed as IWRS directs, expect 2 (W13)
- ☐ New seizure diary dispensed
- ☐ Visit 6 scheduled in 2 weeks and the Day 36 phone call booked

Visit 6, Day 43 (±2 days): end of titration

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs (W3)
- ☐ Single 12-lead ECG (W7)
- ☐ Brief neurologic exam (W6)
- ☐ Blood for hematology and chemistry (W8)
- ☐ Drug accountability (W13)
- ☐ Investigator's tolerability decision. No upward titration after week 6. One 50 mg reduction remains possible through the end of week 8, at an unscheduled visit (W17)
- ☐ Cards dispensed as IWRS directs for the 4 weeks to Visit 7, expect 4 (W13)
- ☐ New seizure diary dispensed
- ☐ Visit 7 scheduled in 4 weeks. Subject told it includes two blood draws 2 to 4 hours apart

Maintenance and end of double-blind: Visits 7 to 9

The dose is fixed after week 8, so these visits are about clean seizure data, timed drug levels and the Visit 9 endpoint assessments.

Visit 7, Day 71 (± 3 days)

The timed samples set the order today: draw on arrival, then do everything else while the clock runs.

- ☐ On arrival: first YKP3089 blood sample, about 8 mL, with the exact draw time recorded (W9)
- ☐ Same draw: concomitant AED levels, at trough if the visit time allows (W9)
- ☐ Date, time and dose of the last YKP3089 dose and of each AED recorded before the draw (W9)
- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ Brief neurologic exam (W6)
- ☐ Drug accountability (W13)
- ☐ Second YKP3089 blood sample, 2 to 4 hours after the first, with the exact draw time recorded (W9)
- ☐ Cards dispensed as IWRS directs, expect 4 (W13)
- ☐ New seizure diary dispensed
- ☐ Visit 8 scheduled in 4 weeks

Visit 8, Day 99 (± 3 days)

- ☐ On arrival: first YKP3089 blood sample and concomitant AED levels, with last-dose details and draw time recorded (W9)
- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs (W3)
- ☐ Brief neurologic exam (W6)
- ☐ Drug accountability (W13)
- ☐ Second YKP3089 blood sample, 2 to 4 hours after the first (W9)
- ☐ Cards dispensed as IWRS directs, expect 4 (W13)
- ☐ New seizure diary dispensed
- ☐ Extension explained again so the subject can decide by Visit 9
- ☐ Visit 9 scheduled in 4 weeks

Visit 9, Day 127 (± 3 days): end of double-blind, subject not entering the extension

This is also the procedure list for early termination. Subjects entering the extension use the Visit 9 list in the open-label section instead.

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ QOLIE-31-P completed by the subject alone, same seizure-timing rule as Visit 3 (W11)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ Single 12-lead ECG (W7)
- ☐ Full neurologic exam (W5)
- ☐ CGIC completed by the investigator, weighing seizure frequency and severity, adverse events and overall function (W11)

- ☐ Urine pregnancy test (W8)
- ☐ Blood for hematology and chemistry, and urine for urinalysis (W8)
- ☐ Drug accountability (W13)
- ☐ Subject's decision on the extension documented (W18)
- ☐ 3 taper cards dispensed as IWRS directs. Every subject gets them, whatever the dose, to protect the blind (W13)
- ☐ New seizure diary dispensed
- ☐ Visit 10 scheduled in 5 weeks, which is 14 days after the last taper dose

Leaving the study: taper, Visit 10 and early termination

Every subject who stops, for any reason, gets the Visit 9 assessments, a taper if clinically warranted, and a follow-up visit 14 days after the last dose.

Early termination before Visit 9

- ☐ Subject brought in as soon as possible and every Visit 9 procedure above completed
- ☐ Reason for withdrawal recorded: consent withdrawn, non-adherence, intolerable adverse event, pregnancy or lost to follow-up (W18)
- ☐ Investigator decides whether a taper is clinically warranted. If so, taper cards dispensed as IWRS directs (W13)
- ☐ Visit 10 scheduled 14 days after the last dose
- ☐ Consent withdrawn: what the subject still agrees to is written down in their words
- ☐ Pregnancy: study drug stopped and the sponsor notified. The protocol does not describe pregnancy reporting, so ask the monitor for the form and timeline
- ☐ Lost to follow-up: every contact attempt documented as your site procedure requires

Taper weeks between Visit 9 and Visit 10

- ☐ Subject knows which card belongs to which week, by the dates written on the cards
- ☐ Subject knows the investigator may adjust or add AEDs during the taper
- ☐ Subject keeps completing the seizure diary and calls for any increase in seizures

Visit 10, Day 162 or 14 days after the last dose (± 3 days)

- ☐ Final seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ Full physical exam. New clinically significant findings are recorded as adverse events (W4)
- ☐ Brief neurologic exam (W6)
- ☐ Serum pregnancy test (W8)
- ☐ Blood for hematology and chemistry, and urine for urinalysis (W8)
- ☐ Final drug accountability: every taper card returned and counted (W13)
- ☐ No study drug dispensed
- ☐ Subject told to report any later event that might be related to the study, and that contraception continues until 30 days after the last dose
- ☐ Contact booked for 30 days after the last dose to close out adverse events (W16)
- ☐ End-of-study record completed (W18)

Open-label extension: Visits 9, 11 to 17 and beyond year one

Subjects who enter the extension skip Visit 10. Windows are ± 2 days for Visits 11 and 12, ± 3 days for Visits 9 and 17, and ± 7 days for the rest.

Visit 9, Day 127 (± 3 days): entering the extension

- ☐ Every item on the double-blind Visit 9 list completed, except the taper cards and the Visit 10 booking
- ☐ Full physical exam added (W4)
- ☐ Inclusion and exclusion criteria re-checked, seizure frequency aside, and signed by the investigator (Eligibility checklist)
- ☐ 2 blinded blister cards and 1 open-label bottle of 50 mg tablets dispensed as IWRS directs (W13)
- ☐ Bottle label completed: subject number, date dispensed, site number, phone contact, investigator name
- ☐ Dosing taught: every morning 4 tablets from the card, plus 2 bottle tablets in week 1 and 4 bottle tablets in week 2. All card tablets are taken
- ☐ Subject told that AED doses stay unchanged until Visit 11
- ☐ New seizure diary dispensed
- ☐ Visit 11 scheduled in 2 weeks

Visit 11, Day 141 (± 2 days): conversion ends

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs (W3)
- ☐ Brief neurologic exam (W6)
- ☐ Drug accountability for both cards and the bottle (W13)
- ☐ Open-label dose set by the investigator: target 300 mg, range 50 to 400 mg (W17)
- ☐ Open-label bottles dispensed as IWRS directs. At 300 mg the subject takes three 100 mg tablets every morning (W13)
- ☐ Subject told that AED changes are now allowed, by the investigator only
- ☐ New seizure diary dispensed
- ☐ Visit 12 scheduled in 2 weeks

Visit 12, Day 155 (± 2 days)

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs (W3)
- ☐ 12-lead ECG (W7)
- ☐ Urine pregnancy test (W8)
- ☐ Blood for hematology and chemistry (W8)
- ☐ Drug accountability, then bottles dispensed as IWRS directs (W13)
- ☐ New seizure diary dispensed
- ☐ Visit 13 scheduled in 12 weeks

Visit 13, Day 239, and Visit 15, Day 407 (± 7 days)

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ Brief neurologic exam (W6)
- ☐ Drug accountability, then bottles dispensed as IWRS directs (W13)
- ☐ New seizure diary dispensed
- ☐ Next visit scheduled in 12 weeks

Visit 14, Day 323 (± 7 days)

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs (W3)
- ☐ Urine pregnancy test (W8)
- ☐ Blood for hematology and chemistry, and urine for urinalysis (W8)
- ☐ Drug accountability, then bottles dispensed as IWRS directs (W13)
- ☐ New seizure diary dispensed
- ☐ Visit 15 scheduled in 12 weeks

Visit 16, Day 491 (± 7 days): one year, or termination from the extension

- ☐ Seizure diary collected and reviewed (W10)
- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ 12-lead ECG (W7)
- ☐ Full physical exam and full neurologic exam (W4, W5)
- ☐ Urine pregnancy test (W8)
- ☐ Blood for hematology and chemistry, and urine for urinalysis (W8)
- ☐ Drug accountability (W13)
- ☐ Investigator documents whether the subject is benefiting from YKP3089 (W18)
- ☐ Continuing: bottles and a new diary dispensed, next visit scheduled in 3 months
- ☐ Leaving: taper bottles dispensed as IWRS directs if a taper is needed. Visit 17 scheduled 2 weeks after the last dose

Beyond year one: Visit 18 onward, every 3 months (± 7 days)

- ☐ Every visit: seizure diary, adverse events, concomitant medications, C-SSRS, vital signs, drug accountability, bottles dispensed
- ☐ Every 6 months from Visit 16, add: weight, brief neurologic exam, urine pregnancy test, hematology and chemistry
- ☐ Every 12 months from Visit 16, add: full physical exam, 12-lead ECG, urinalysis
- ☐ Stopping after year one: subject seen as soon as possible, Visit 16 procedures done under the next visit number, then Visit 17

Visit 17, 14 days after the last dose (±3 days): end of study

- ☐ Adverse events and concomitant medications (W15, W14)
- ☐ C-SSRS, Since Last Visit version (W11)
- ☐ Vital signs and weight (W3)
- ☐ Full neurologic exam (W5)
- ☐ Serum pregnancy test (W8)
- ☐ Blood for hematology and chemistry, and urine for urinalysis (W8)
- ☐ Final drug accountability: every bottle returned and counted (W13)
- ☐ No study drug dispensed
- ☐ Contact booked for 30 days after the last dose to close out adverse events (W16)
- ☐ End-of-study record completed (W18)

Unscheduled visits and calls

Either the investigator or the subject can ask for an extra visit or call at any time; record the reason, any adverse events and every procedure performed, in source and in the eCRF.

- ☐ Reason for the contact, date, time and who was present recorded (W16)
- ☐ Adverse events and concomitant medications reviewed (W15, W14)
- ☐ Rash: subject seen promptly, hematology and chemistry drawn, investigator evaluates for drug hypersensitivity (W8)
- ☐ Dose reduction: rules on the Synopsis tab checked, all cards returned, replacement cards issued by IWRS (W17, W13)
- ☐ Suspected pregnancy: interim pregnancy test (W8)
- ☐ Suspected concomitant AED toxicity: AED level drawn and sent to the local lab (W9)
- ☐ Exclusionary lab value at Visit 1: one repeat sample for confirmation (W8)
- ☐ QOLIE-31-P postponed because of a recent seizure: completed once the subject has recovered (W11)
- ☐ Investigator informed and note signed (W19)